



AI-Powered Clinical Chart Intelligence

Transforming how hospitals read, summarize,
and act on patient data

THE PROBLEM

Every Shift Change Is a Patient Safety Risk

30-60

Min/Handoff

Nurses spend 30-60 min reading charts, notes, labs, and orders from multiple providers before safely taking over care.

2-3

Hrs/Day Lost

Hospitalists covering 15-20 patients waste 2-3 hours daily on chart review — 25-35% of their shift not seeing patients.

80%

Error Rate

Joint Commission: 80% of serious medical errors involve miscommunication during care transitions and handoffs.

\$1.7B

Annual Cost

US hospitals lose \$1.7B annually to preventable errors during shift transitions: malpractice, readmissions, extended stays.

The status quo is unsustainable.

63% of physicians report burnout. Documentation is the #1 cause of hospital staff turnover.

THE SOLUTION

AI That Reads Every Chart So Your Team Doesn't Have To

1

Instant Shift Summaries

AI ingests every note from every provider: attending physicians, residents, nurses, specialists, therapists. Produces organized, problem-based summaries in seconds. Flags critical changes, new orders, and trending labs. What took 45 minutes now takes 10 seconds.

2

Smart SBAR Handoffs

One-click generation of SBAR-formatted handoff reports for your entire patient census. Meets Joint Commission standards. Standardizes quality whether the outgoing nurse is meticulous or rushed. Printable and shareable across units.

3

Clinical Intelligence Chat

Natural language interface to every patient chart. "When was the last potassium check?" "Summarize the surgery note from Tuesday." "What antibiotics has she been on?" Instant answers from the full chart, zero chart diving.

Average chart review time: before vs. after ChartIQ

Built for Every Stakeholder

Nurses

Save 30-60 minutes per shift change. Focus on patient care instead of reading dozens of notes. Reduces cognitive overload during critical transitions. Less burnout, better retention.

Hospitalists & Physicians

Cover 15-20 patients efficiently without drowning in documentation. Instant summaries when called to consult on unfamiliar patients. Night and weekend coverage becomes manageable.

Hospital Administrators

Reduce documentation-related overtime costs. Improve patient safety scores and CMS Star Ratings. Meet Joint Commission handoff requirements. Reduce malpractice exposure.

Health Systems & Networks

Standardize handoff quality across all facilities. Aggregate clinical intelligence. Reduce average length of stay through faster clinical decisions. Benchmark across hospitals.

Priority Targets

Community hospitals (50-200 beds) — fastest sales cycle
Academic medical centers — larger contracts

HCA (186 hospitals), Tenet (60), CommonSpirit (140)
Baptist Health, Memorial Healthcare, Jackson Health

Core Feature Set

AI Shift Summary

Reads all notes from all providers. Organizes by active problem. Flags critical lab trends, new orders, medication changes, and code status updates.

SBAR Handoff Generator

One-click Joint Commission-compliant SBAR handoff for entire census. Situation, Background, Assessment, Recommendation for each patient.

Patient Clinical Chat

Natural language interface to any patient chart. Ask any question, get instant answers from the full medical record. Available per-patient and globally.

Voice Dictation Notes

Speak notes directly into the system. Real-time transcription. Multiple note types: progress, assessments, procedures. Hands-free at bedside.

Vitals & Labs Dashboard

Real-time vitals charts with Q4H trending. Lab results with automatic flagging. CBC, BMP, cardiac markers, coagulation panels. Medication reconciliation.

Medical Image Gallery

Upload and organize X-rays, MRIs, CTs, ultrasounds. Category badges, drag-and-drop upload, lightbox viewing. Attach images to handoff reports.

Massive Addressable Market

6,090

US Hospitals

Community, academic,
and critical access

924K

Staffed Beds

Each bed = monthly
recurring subscription

\$110M

ChartIQ TAM/yr

At \$10/bed/month
across all US hospitals

\$500M+

Combined TAM

ChartIQ + KOSIQ
population health

Why Now?

Physician Burnout Crisis

63% of physicians report burnout. Documentation is the #1 cited cause. Hospitals are desperate for solutions.

Joint Commission Mandates

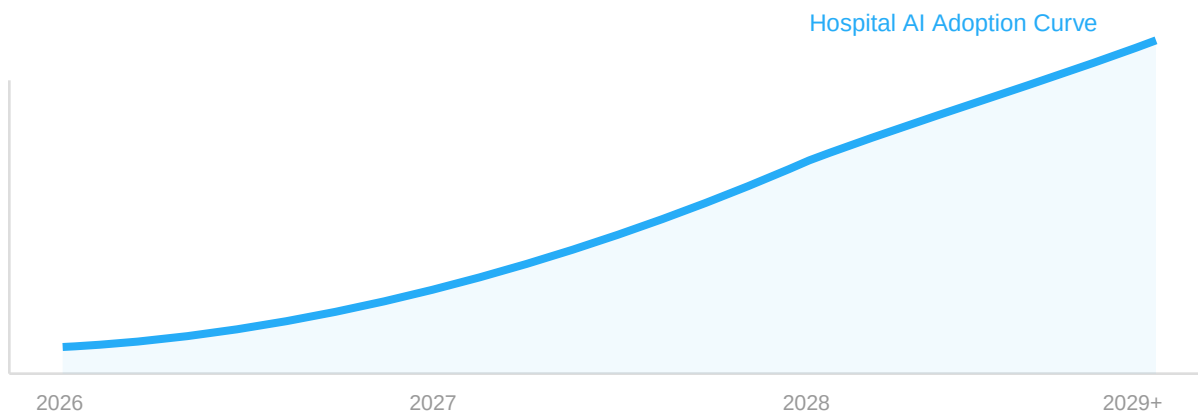
Accreditation requires standardized handoff communication. Most hospitals still use paper or verbal handoffs.

AI Acceptance in Healthcare

Hospital IT spending on AI grew 47% in 2025. Epic, Cerner, MEDITECH all building AI ecosystems.

Staffing Shortages

US nursing shortage projected at 1.2M by 2030. Every minute saved on documentation goes to patient care.



BUSINESS MODEL

Per-Bed, Per-Month SaaS

\$15

/bed/month

Small Hospitals (under 100 beds)

\$18K-\$180K/yr per hospital

Community hospitals, critical access, rural

\$12

/bed/month

Medium Hospitals (100-300 beds)

\$144K-\$432K/yr per hospital

Regional medical centers, teaching hospitals

\$8

/bed/month

Large Hospitals (300+ beds)

\$288K+ per hospital

Academic medical centers, HCA/Tenet facilities

Additional Revenue

\$5K-\$25K

Implementation Fees

One-time setup, EHR integration, training

90%+

Gross Margins

AI inference ~\$0.02/summary, minimal infra

120%+

Net Revenue Retention

Expand from pilot units to full facility

60-90 days

Sales Cycle

Low-risk pilot, prove ROI, then expand

REVENUE PROJECTIONS

Three-Year Growth Path

	Conservative	Moderate	Aggressive
Hospitals	5	10	15
Avg Beds	150	150	200
Avg Price	\$10/bed	\$12/bed	\$12/bed
Monthly Rev	\$7,500	\$18,000	\$36,000
Setup Fees	\$25K	\$75K	\$150K
Year 1 Total	\$115K	\$291K	\$582K

Growth Trajectory

Year 2 \$650K - \$3.15M 25-100 hospitals — Regional expansion, enterprise contracts
Year 3 \$3.7M - \$6.1M 100-200 hospitals — National scale + KOSIQ upsell revenue

The Trojan Horse Strategy

ChartIQ is the wedge. KOSIQ is the platform.



The Math: One Hospital, Two Products

ChartIQ Revenue

300 beds x \$10/mo = \$36K/year

+

KOSIQ Revenue

20K lives x \$1 PMPM = \$240K/year

\$276K - \$516K per hospital per year

At 50 hospitals with 10 KOSIQ conversions: \$1.3M + \$2.4M = \$3.7M+ ARR

COMPETITIVE LANDSCAPE

We Read Charts. They Create Notes.



		Chart Summary	Cross-Provider Intel	Handoff Generation	Clinical Chat	Population Health
	Seed					
Abridge	\$212M	☒	☐	☐	☐	☐
Suki	\$400M+	☐	☐	☐	☒	☐
DeepScribe	\$60M+	☐	☐	☐	☐	☐
Nabla	\$44M	☐	☐	☐	☐	☐

Competitors: Note CREATION

Ambient listening during patient encounters.
Generates new notes from conversations. Solves
documentation input.

ChartIQ: Chart INTELLIGENCE

Reads and synthesizes ALL existing notes from
ALL providers. Summaries, handoffs, chat. Solves
information consumption.

12-Month Execution Plan

1

Months 1-3

Foundation

HIPAA compliance certification (shared with KOSIQ, ~\$75K). SOC 2 Type I. Build EHR integration layer for Epic FHIR, Cerner HealthIntent, MEDITECH Expanse. Hire healthcare sales rep for South Florida.

2

Months 4-6

Pilot & Prove

Launch pilots at 2-3 South Florida hospitals. Targets: Baptist Health (11 hospitals, 2,500 beds), Memorial Healthcare (6 hospitals, 1,900 beds). Price at \$10/bed to minimize friction. Collect ROI metrics.

3

Months 7-9

Validate & Expand

Publish case studies ("Nurses saved 45 min/shift, zero handoff errors in 90 days"). Expand within pilot systems. Sign 2-3 additional hospitals. Add voice summarization feature.

4

Months 10-12

Scale

Reach 8-12 hospitals across South Florida. Present at HIMSS Global Health Conference. Begin enterprise conversations with HCA, Tenet, CommonSpirit. List on EHR app marketplaces.

24-Month Growth Plan

Regional Expansion

Expand into Atlanta, Houston, Dallas, broader Southeast. 3 regional sales reps. Target 50+ hospitals. Establish ChartIQ as standard-of-care for shift handoffs in community hospitals.

Predictive Clinical Alerts

AI-powered patient deterioration detection. Analyze vitals, labs, and nursing notes to flag patients likely to code or transfer to ICU within 6-12 hours. Predictive intelligence.

Enterprise Health Systems

HCA: 186 hospitals, 46K beds = \$4.4M/yr.
Tenet: 60 hospitals = \$1.5M/yr. CommonSpirit: 140 hospitals = \$3.2M/yr. One enterprise deal replaces 20+ individual sales.

KOSIQ + ChartIQ Bundle

Unified healthcare AI platform. Clinical operations + population health analytics. Single contract, single compliance stack. Two revenue streams per customer. Avg deal: \$300-500K/yr.

100+ hospitals, national footprint, enterprise contracts, bundled platform revenue

THE TEAM

Clinical Expertise Meets AI Execution



Dr. JD Suarez

Founder & Chief Medical Officer

30+ years in managed care medicine. Chief Medical Officer of a healthcare group in South Florida managing thousands of Medicare Advantage patients. Deep clinical credibility and relationships with hospital administrators, health system executives, and payer organizations across the region.

Dr. JD brings domain expertise, clinical validation, and industry relationships that make ChartIQ credible to hospital buyers from day one.



David Suarez

Co-Founder & Chief Product Officer

AI product builder who took both KOSIQ and ChartIQ from concept to working product in weeks. Combines technical AI expertise with deep understanding of healthcare workflows learned directly from clinical stakeholders.

David brings product vision, rapid execution, and the technical infrastructure that turns clinical insight into software hospitals actually use.

Father-Son Founding Team

A practicing CMO who lives the problem daily + an AI product builder who ships solutions in days, not months.

ChartIQ

Part of the KOSIQ Family of Healthcare AI Products

- 1 Shared infrastructure and compliance stack — one investment covers both products
- 2 Two distinct revenue streams: clinical operations + population health analytics
- 3 Two paths into every hospital: daily workflow tool opens the door for enterprise analytics
- 4 Combined TAM: \$500M+ across 6,000+ US hospitals

Let's talk

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